

[illegible]

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: هناء سند رشاد سيد National ID: 27901212101221	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Name : Hanaa Sanad Rashad Said

Date : 11-05-2026 07:37

Branch : Dokki

Medical No. : 371799

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

Nuclear Medicine Report: Skeletal Scintigraphy

Clinical history and Indication: 47-year-old female patient with history of breast cancer. Follow-up for metastatic workup due to specific bony aches.

Technique:

Anterior and posterior whole-body planar scans were acquired 2 hours post-injection of the tracer (^{99m}Tc -MDP).

Reference Study: Comparison made with previous scans dated April 13, 2022, February 25, 2025, and September 13, 2025.

Findings:

Skeletal Metastases:

Multiple foci of enhanced tracer uptake remain evident at the following sites:

- Axial Skeleton: Skull, sternum, and multiple levels of the spine.
- Appendicular Skeleton: Right shoulder, right humerus, and proximal bilateral femora.
- Pelvis: Right sacroiliac region and general pelvic bones.
- Thorax: Bilateral ribs.

Stable Comparison: When compared to previous studies dated 25/02/2025 and 13/09/2025, the number and intensity of the osseous lesions show no significant interval change.

Benign Findings: Degenerative changes are noted in the spine and major joints.

Impression:

Disease Status: Stationary course of the previously described multiple metastatic osseous lesions.

Comparison: No newly developed lesions or significant progression identified relative to the prior examinations.

Recommendation: Follow up imaging as clinically indicated.

Dr. Alaa Nagy

جمهورية مصر العربية
بطاقة تحقيق الشخصية

هنا
سند رشاد سيد
قرية غمازة الصغرى
مركز الصف - الجيزة

١٩٧٩/٠١/٢١ ٢٧٩ ٠١٢١ ٢١ ٠١٢ ٢١

J10980583

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أوميا

٢٠٢٢/٠٤ ٢٧٩ ٠١٢١ ٢١ ٠١٢ ٢١

ربة منزل

أنثى
مسلمة
متزوجة

عبد العليم محمد عبد العليم حسين

البطاقة سارية حتى ٢٠٢٩/٠١/٠٢

١

Name: _____ MR. : 1114	هناك سند رشاد
Detailed Medical Report	
السيد	
تحتية طبية وبعد،،	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمناظرة وجد انها 2021: Oligometastatic breast cancer with D11 bone metastases treated with neoadjuvant chemotherapy (AC*4 + TAXOER*4), radiation therapy, and mastectomy. The patient received NOLVADEX for 3 years. A bone scan in March 2025 showed progressive disease; the multidisciplinary team recommended FEMARA, IBRANCE, and ZOLADEX. Ongoing management includes oligometastatic breast cancer treated with IBRANCE, FEMARA, and ZOLADEX. Laboratory evaluation was performed prior to initiating IBRANCE. The patient continues on FEMARA and ZOLADEX as supportive therapy. The patient presents with metastatic breast cancer to bone and is on aromatase inhibitor therapy with IBRANCE. A CT scan of the chest, abdomen, and pelvis is pending, as is a bone scan. The patient was seen today for initiation of FEMARA and is awaiting approval for IBRANCE therapy. Bone scan results are pending. The patient is awaiting approval for IBRANCE therapy in conjunction with FEMARA and supportive care. Bone scan in May 2026 shows stable disease on FEMARA and supportive IBRANCE 125 mg. Approval for ZOMETA is pending after dental clearance. Bone scan in May 2026 shows stable disease on FEMARA and supportive IBRANCE 125 mg. Approval for ZOMETA is pending after dental clearance. Bone scan in May 2026 shows stable disease on FEMARA and supportive IBRANCE 125 mg. Approval for ZOMETA is pending after dental clearance. The next visit will occur once IBRANCE is approved. Multidisciplinary team discussion in March 2025 reaffirmed the use of FEMARA, IBRANCE, and ZOLADEX for the patient's metastatic breast cancer. Dental assessment: the patient will address dental issues prior to initiating IBRANCE 125 mg after a complete blood count and FEMARA. The next follow-up is scheduled in one month.	
تحريراً في ,	
التاريخ :	20-09-2026
الطبيب المعالج :	أسماء محمد مصطفى نصر
يعتمد ,	

Patient Name : الأمستلاه / هناء سند رشاد سيد
Patient No. : 27901212101221
Path No. : 2404/21
Sex : Female
Age : 42 Y
Referring Doc :

Request No. : 139885
Request Date : 12/10/2021
Verification Date : 18/10/2021
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient

Pathology Report

Clinical Data:

Right breast invasive duct carcinoma grade II. Post chemotherapy.

Nature of the specimen:

Right MRM

Gross Examination:

Right MRM specimen measured 30x19x3cm, skin covering measured 23x15cm, showing scar measured 6cm long. Nipple and areola are unremarkable grossly. Cut section revealed tumour bed site 2.5x2x1cm. Axillary dissection showed 9 lymph nodes up to 1cm.

Microscopic examination:

- Histological type: No remnants of invasive tumour tissue.
- Grade: N/A
- Angioinvasion: No
- Ductal carcinoma in situ: Foci of DCIS.
- Lymph node involvement: All lymph nodes are free (0/9)
- Resection Margins: Free deep margin
- Histologic response to therapy: Complete pathological response
- Additional pathologic findings: Free nipple and areola

Diagnosis:

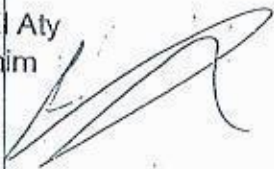
Right MRM:

No remnants of invasive tumour tissue.

Complete pathological response.

Free all lymph nodes (0/9), nipple and areola.

Prof. Dr. Gamal Abd-El Aty
Dr. Abdulrahman Ibrahim



Patient Name: Hanaa Saned Rashad Sayed	Date : 24/12/2024
Exam : C.T ABDOMEN & PELVIS	Age : 46 Year(s)
Medical ID: 780/2021	Pat. ID: 21938

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED:**

Compared to study dated 11/8/2024,

1. Chest:

- Right MRM with no sizable masses.
- Grossly clear both lung fields. No pulmonary nodules or masses
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged in size with no current hepatic focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

PATIENT NAME: HANAA SANAD RASHAD SAYED	PATIENT ID #: 2021148257
GENDER: Female	ORDER #: 1020492
AGE: 42 y	DATE OF SERVICE: 28-Mar-2021 12:35 PM

CAP

Report:

MRI EXAMINATION OF THE PELVIS

Clinical History:

- Known breast cancer patient.

Technique of examination:

- Axial T1 & T2 WIs "Plate 1 & 2".
- Coronal T1 & T2 WIs "Plate 3 & 4".
- Post Gadolinium DTPA Axial & Coronal T1WIs "Plate 5-6".

MRI Findings:

- Average AVF uterus showing preserved junctional zone and average endometrial thickness. No sizable myometrial focal lesions.
- Normal MRI appearance of the cervix and vagina.
- Right ovarian dominant follicle/small cyst. Otherwise, no adnexal solid or cystic masses.
- Smooth filling of the urinary bladder with no mural thickening or diverticular outpouching.
- Small osseous foci of persistently low MR signal are seen at the right femoral intertrochanteric region and left iliac bone, likely bone island.
- Small bone cyst is seen at the posterior column of the right acetabulum.
- No marrow infiltration.

Conclusion:

- Small osseous foci of persistently low MR signal are seen at the right femoral intertrochanteric region and left iliac bone, likely bone island. Otherwise, unremarkable MRI study of the pelvis.

Reported By,

Dr. Mahmoud Ibrahim, MSc
Dr. Kareem Moussa, MD

29-Mar-2021 01:20 PM

سامو جراف

Thank you for referring this case,
Patient name : hanaa sanad
Date : 10-2-2021

Bilateral soft tissue mamography and breast sonography :

- AC R B .
- Normal skin thickness and contour .
- scattered mixed densities against a homogenous fatty background.
- Left breast : No spiculated masses or pathological microcalcification.
- Right breast shows : multile fairly defined isodense lesions located at the upper outer quadrenrent no microcalcifications, Complementary sonography shows multiple hypoechoic irregular lesions , the largest measures 14 x10 mm with internal vascularity , no posterior shadwing , located at 9 oclock , one axillary tail lymph node is seen measures 20 x13 mm .
- Left breast : no solid or cystic masses, architecture disruption or abnormal vascularity.
- Echogenic mammary parenchyma and prominent lactiferous ducts.
- Bilateral axillary lymph nodes with preserved fatty hilum.
- **OPINION :**
- Right breast multiple solid lesions ? BIRADS IV c for biopsy and hystopathological correlation.
- Left breast : normal study, BIRADS I.

KINDLY ACCEPT MY BEST WISHES,
DR/ sanaa mohamed ,msc



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : هناء صند رشاد صويد

Sex:

3- Medical number:

Female

83034889

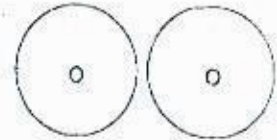
Old Med :

Dis Med :

780-2021

2-ID number: 27901212101221

Patient Age : 46 Y



Family history

Family history

Phone

Phone

01150029883

BMI

BMI

REASON OF MDT

REASON OF MDT
ACCORD

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2025-03-18

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status

Initial STAGE

Indic STAGE

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
BX COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T	N	M
		1 BONE

Sonomammography

PATIENT HISTORY

RT MRM --> received NACTA the hormonal for 3 yrs
been scan show multiple met osseous lesion at the mentioned site (skull, sternum, rt
shoulder, rt hilum) with progressive course and newly developed
pt received palliative RTH

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

start femara, IBRANCE, zoladex acc E2, FSH, LH LEVEL

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY /s/ خالد عبد العزيز	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Nuclear Medicine Unit

وحدة الطب النووي

Hanaa Sanad Kashad

Date: 25/02/2025

Tc99m MDP SKELETAL SCINTIGRAPHY

History

- A 46-year-old patient with history of breast cancer; referred for follow up.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Compared to the last bone scan dated 13/04/2022; the current study showed progressive course with multiple newly developed osseous lesions currently seen involving skull, sternum, right shoulder, right humerus, different levels of the spine; as well as; right sacro-iliac region and proximal left femur.
- Degenerative changes are noticed at different levels of spine and large joints.

IMPRESSION

- Multiple metastatic osseous lesions at the aforementioned sites with progressive course and newly developed ones.

Appreciated,

Dr.Hossam Sekkina, M.Sc.

**Dr. Alaa Nagi, MD
Nuclear Medicine**



25
الدائرة

Hotline

02 21 29 2000

www.ahramscan.com

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فرع فيصل : 434 شارع الملك فيصل (فوق مستشفى تيارك للأطفال) الجيزة
فرع الدقي : 92 شارع التحرير - برج ساريدار الطبي - الدور الثالث
فرع شبرا : 128 شارع شبرا - أمام قسم الساحل - بجوار ستيرال شبرا
فرع أكتوبر : رنا مول بعد الحصري والتوحيد والنور فوق سوپر ماركيت أولاد رجب - الدور الثاني

[[DOCTYPE:BONE_SCAN]]

Name : hanaa sanad rashad

Date : 07-03-2026 03:40

Branch : Portable

Medical No. : GRI-27901212101221

Ref.Physician Prof : ct cap+c

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN
AND PELVIS REVEALED:**

I- CHEST:

- o Evidence of right mastetocmy.
- o Clear both lungs showing no sizable pulmonary nodules, consolidation or cavitation.
- o No pathologically enlarged hilar or mediastinal lymph nodes.
- o No plural or pericardial collection.
- o No gross cardiomegaly

II- ABDOMEN AND PELVIS:

- o Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Normal C.T appearance of the spleen, pancreas, both kidneys, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No pathologically enlarged abdominal or pelvic lymphadenopathy.
- o No ascites.

III- Skeletal survey:

- o Multiple vertebral and iliac bone sclerotic bony lesions

Opinion:

- ❖ Hepatomegaly.
- ❖ Multiple vertebral and iliac bone sclerotic bony lesions for bone scan correlation.

Dr. Nourhan Mohamed,MD

08/03/2026 05:00 AM

Patient Name: Hanaa Saned Rashad Sayed	Date: 24/12/2024
Exam: C.T ABDOMEN & PELVIS	Age: 46 Year(s)
Medical ID: 780/2021	Pat. ID: 21938

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- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

anaa Sanad Rashad Said

26-09-08

Prof.Dr. no doctor
Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE:

Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported right breast carcinoma underwent right modified radical mastectomy. No available previous DICOM images for comparison.

(I) Chest:

Both Lung Fields are showing no nodules, consolidation or collapse.

- Both Lung Fields are showing no nodules, consolidation or collapse.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening, or masses were seen
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal appearance of the chest wall.
- Normal thoracic vertebral bodies scattered sclerotic foci and T11 & T12 vertebral body lytic lesions with bony sclerotic margins with T11 body partial collapse with noted rib multiple tiny sclerotic lesion mainly anteriorly.

(II) Abdomen and pelvis:

- ☐ The liver is enlarged in size showing diffuse hypo dense yet homogenous CT density , no focal lesions
- ☐ Patent and homogenously enhancing portal vein.
- ☐ No intra or extra hepatic biliary radicles dilatation.
- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- ☐ Spleen is showing Normal CT appearance.
- ☐ Regular filling of the urinary bladder showing no stones, masses .
- ☐ Unremarkable other pelvic organs.
- ☐ No para-aortic or pelvic lymphadenopathy.
- ☐ No ascites.
- ☐ Noted L1 heterogeneous vertebral body sclerosis and L2 to L5 showing multiple sclerotic foci.
- ☐ Noted right neck femur and sacral and pelvic iliac bone multiple sclerotic lesions mainly in iliac wings.

Patient's Name:

Hanaa Sanad Rashad Said

Date:

2026-09-08

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

OPINION:

- ✓ Fatty hepatomagely.
- ✓ Thoraco-lumbar vertebral bodies lytic & sclerotic lesions with multiple tiny ribs sclerotic foci and right neck femur and sacral and pelvic iliac bone multiple sclerotic lesions mostly are metastatic foci for PET CT correlation.
- ✓ No other Appreciable pulmonary or Abdominopelvic lesions.

Best regards,

Dr. Ayman Shawky, FEBR